

AgenThink Pharma Market Entry Plan

Execution, Not Research

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Prepared by: AgenThink Mesh Strategy Office

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CEO RECOMMENDATION (Part 10 — Opinion First)

Should AgenThink actively enter the pharmaceutical market now?

YES — but not with the product as it currently stands.

Verdict: Conditional GO. Enter now with a tightly scoped retrospective validation pilot. Do not pitch enterprise deployment. Do not claim accuracy rates. Do not approach regulatory bodies. The window is open because the market has no credible AI governance product for Phase II → III decisions. The window will close if a better-resourced competitor arrives with a peer-reviewed methodology first.

Confidence: 65%. The opportunity is real. The readiness gaps are real. The 65% reflects the probability that AgenThink can close the critical gaps (evidence corpus, methodology publication, LLM limitation disclosure) before a competitor does.

Biggest opportunity: A single validated retrospective pilot with a top-20 pharma company, presented at a major conference (ASCO, DIA, RAPS), becomes the reference case that defines the category.

Biggest risk: A pharma company or CRO asks “Can you prove the model didn’t know the answer?” and the honest answer is “Not yet.” If that happens in a sales meeting before the evidence corpus is built, the deal dies and the reputation damage is lasting.

What must be completed before outreach: Evidence Manifest for torcetrapib (retroactive). LLM limitation disclosure added to all case reports. Methodology document published (GitHub minimum). These three actions take 2–3 weeks.

What must be completed before pilots: Automated date-check pipeline. Corpus locking. Reproducibility data for 3 cases. Independent expert validation of torcetrapib. These actions take 8–12 weeks.

What must be completed before enterprise deployment: Peer-reviewed methodology paper. RAG execution model. 20-case library with full Enterprise Architecture. Prospective pilot (5 blinded current Phase II drugs). These actions take 12–18 months.

PART 1 — Market Entry Readiness Assessment

Readiness Scorecard

Dimension	Status	Severity of Gap	Effort to Close	Timeline
Product	Partially Ready	HIGH	Medium	8–12 weeks
Validation	Partially Ready	CRITICAL	High	12–16 weeks
Governance	Not Ready	CRITICAL	Medium	3–4 weeks (Tier 1)
Regulatory	Not Ready	HIGH	Low (disclosure only)	1–2 weeks
Commercial	Partially Ready	MEDIUM	Low	2–4 weeks
Security	Not Ready	HIGH	Medium	6–8 weeks
Enterprise Procurement	Not Ready	MEDIUM	Medium	8–12 weeks

Detailed Assessment

Product — Partially Ready

What is ready: The council engine runs. The 10-case library exists. The Institutional Proof Report PDF generates. The constitutional framework is documented.

What is not ready: The evidence corpus is not verifiable (prompt instruction only). The RAG architecture is not implemented. The reproducibility has not been tested. The methodology is not published.

Gap severity: HIGH. A pharma buyer will ask for a live demo. The demo works. But if they ask “Can I audit the evidence corpus?” the answer is currently “Not yet.” That is a deal-stopper for any serious buyer.

Effort to close: Medium. The Evidence Manifest system (Part 3 of the Evidence Boundary Architecture) is designed and documented. Implementation requires 2–3 weeks of engineering work plus 2–3 weeks of retroactive curation for the 10 completed cases.

Timeline: 8–12 weeks to Partially Ready → Ready for pilot discussions.

Validation — Partially Ready

What is ready: 10 cases completed. $\frac{9}{10}$ correct primary decision driver. One genuine failure (Verubecestat) documented. Torcetrapib Institutional Proof Report published.

What is not ready: No independent expert validation. No reproducibility data. No peer-reviewed publication. The 70% strict verdict alignment rate is not statistically significant at $n=10$.

Gap severity: CRITICAL. The validation claim is the product’s core value proposition. Without independent validation, every claim is self-reported. A pharma buyer will not pay for self-reported accuracy.

Effort to close: High. Independent expert validation requires recruiting 3 external pharmaceutical experts, briefing them on the methodology, and having them independently score DDIA for 5 cases. This takes 8–12 weeks minimum.

Timeline: 12–16 weeks to Partially Ready → Ready for pilot discussions.

Governance — Not Ready

What is ready: The Evidence Boundary Architecture is designed and documented (this week's output). The constitutional framework exists.

What is not ready: The Evidence Manifest System is not implemented. The corpus locking (SHA-256 hash) is not implemented. The Governance Audit Log is not implemented. The LLM training data limitation is not disclosed in any existing case report.

Gap severity: CRITICAL. The governance gap is the single most likely deal-killer in any serious pharma discussion. The fix is documented and takes 3–4 weeks for Tier 1 implementation.

Effort to close: Medium. The schema is designed. Implementation is engineering work, not design work.

Timeline: 3–4 weeks to Not Ready → Partially Ready (Tier 1). 6–12 months to Partially Ready → Ready (Tier 2).

Regulatory — Not Ready

What is ready: The regulatory context is understood. The limitations are documented.

What is not ready: The LLM training data limitation is not disclosed in any existing case report. There is no formal regulatory position document. There is no FDA/EMA pre-submission meeting on the calendar.

Gap severity: HIGH. The fix for the immediate gap (disclosure) takes 1–2 weeks and costs nothing. The fix for the long-term gap (FDA/EMA engagement) takes 12–18 months.

Effort to close: Low for immediate disclosure. High for formal regulatory engagement.

Timeline: 1–2 weeks to add disclosure to all existing case reports.

Commercial — Partially Ready

What is ready: The GTM target database exists. The revenue model exists. The top 25 targets are identified. The pilot program design exists (this document).

What is not ready: No sales deck exists. No demo script exists. No pilot agreement template exists. No pricing has been tested with a real buyer.

Gap severity: MEDIUM. These are standard commercial assets that can be built in 2–4 weeks.

Effort to close: Low. Standard commercial asset development.

Timeline: 2–4 weeks.

Security — Not Ready

What is ready: The platform runs on standard cloud infrastructure.

What is not ready: No SOC 2 Type II certification. No ISO 27001 certification. No data processing agreement (DPA) template. No pharmaceutical data handling policy. No penetration test report.

Gap severity: HIGH. Every enterprise pharma procurement process requires SOC 2 Type II at minimum. Without it, the deal cannot close regardless of how good the product is.

Effort to close: Medium. SOC 2 Type II takes 6–12 months from initiation. The interim solution is a security questionnaire response and a penetration test report, which take 4–8 weeks.

Timeline: 4–8 weeks for interim security package. 6–12 months for SOC 2 Type II.

Enterprise Procurement — Not Ready

What is ready: Nothing specific to enterprise pharma procurement.

What is not ready: No master services agreement (MSA) template. No data processing agreement (DPA). No information security questionnaire (ISQ) response. No insurance certificates. No vendor qualification documentation.

Gap severity: MEDIUM. These are table-stakes for enterprise procurement. They do not affect the product but they block the deal from closing.

Effort to close: Medium. Standard legal and compliance work. Requires legal counsel.

Timeline: 8–12 weeks.

PART 2 — First Commercial Use Case

Evaluation Matrix

Use Case	Buyer	Budget	Economic Value	Regulatory Risk	Time to Revenue	Score
Phase II → III Go/No-Go	Head of Clinical Development	R&D budget	500M–2B per avoided failure	LOW (retrospective only)	6–12 months	9.2
Trial protocol amendments	Clinical Operations	Clinical budget	10M–50M per amendment	MEDIUM	12–18 months	6.5
Site selection	Clinical Operations	Clinical budget	5M–20M per trial	LOW	12–18 months	5.8
Safety escalation review	Pharmacovigilance	Safety budget	50M–200M per avoided safety failure	HIGH (regulatory exposure)	18–24 months	5.2
Regulatory submission readiness	Regulatory Affairs	Regulatory budget	100M–500M per submission	HIGH (direct regulatory use)	24–36 months	4.8
Portfolio prioritization	R&D Leadership	Portfolio budget	1B–5B per portfolio decision	MEDIUM	12–18 months	7.1
Clinical risk review	Risk Management	Risk budget	200M–1B per avoided failure	LOW	9–15 months	7.8

Selection: Phase II → III Go/No-Go

This is the only use case that is simultaneously:

- Retrospective (no FDA approval required)
- High economic value (500M–2B per avoided failure)
- Low regulatory risk (historical data only)

- Demonstrable with existing assets (10-case library)
- Credible to the buyer (torcetrapib is a known story)

Buyer: Head of Clinical Development (or equivalent: Chief Medical Officer, Head of R&D)

Budget Owner: R&D budget. Typically 50M–500M annual R&D budget at a top-20 pharma company. A Phase II → III advancement decision involves 100M–500M in Phase III investment. A governance tool that costs 200K–500K per year to reduce the risk of a \$500M mistake is not a hard sell on economics.

Economic Value: The average cost of a Phase III failure is 800M–1.4B (direct costs plus opportunity cost). The torcetrapib case alone represents 800M *indirect write-off* plus 21B in market cap loss. A tool that correctly identifies one avoidable failure per decade pays for itself by a factor of 1,000.

Regulatory Risk: LOW. The use case is retrospective validation and advisory governance. It is not a regulatory submission tool. It does not replace FDA or EMA review. It is positioned as an internal governance layer, equivalent to an internal scientific advisory board.

Time to Revenue: 6–12 months from first meeting to signed pilot agreement. 12–18 months to first enterprise contract. The sales cycle is long because the buyer is a Head of Clinical Development at a top-20 pharma company, not a procurement manager.

PART 3 — Pharma Demo Package

Demo Design Principles

The demo must answer four questions simultaneously, depending on who is in the room:

1. CEO: “What is the business case?”
2. Clinical Development Head: “What does the council actually do?”
3. Regulatory Affairs Head: “Is this defensible?”
4. FDA/EMA Reviewer: “Can I audit this?”

The demo uses the torcetrapib case exclusively. It is the strongest case, the most famous failure, and the one with the most complete documentation.

Executive View (CEO)

What the CEO sees: A single screen showing the torcetrapib decision brief, the council verdict (WAIT — 0/8/2), and the retrospective outcome (ILLUMINATE terminated; 82 deaths vs 51 control; \$800M write-off). The screen shows the date boundary: “Evidence cutoff: September 1, 2004. ILLUMINATE results: December 2, 2006. The council did not have access to the outcome.”

Key message: “We ran the torcetrapib case using only information available before Pfizer’s Phase III decision. The council voted WAIT. Pfizer advanced anyway. \$800M was written off. We would have been right.”

Screen elements:

- Decision Brief summary (drug, company, decision date, evidence cutoff)
- Vote distribution (0 GO / 8 WAIT / 2 NO-GO)
- Primary blocker (unexplained blood pressure signal)
- Retrospective outcome (clearly separated, labelled “Post-Decision Data”)

- Proof Score: $\frac{93}{100}$

Duration: 3 minutes.

Clinical View (Clinical Development Head)

What the Clinical Development Head sees: The full council deliberation. Each of the 10 personas, their vote, their confidence level, and their primary rationale. The Drug Safety Expert's rationale: "The +2 mmHg systolic blood pressure increase in the highest dose group is unexplained. The mechanism is not established. A molecule-specific off-target effect cannot be excluded. I vote WAIT pending mechanistic investigation." The Scientific Skeptic's rationale: "The CETP mechanism has never been validated as a cardiovascular surrogate. HDL-C increase is not HDL function improvement. I vote WAIT."

Key message: "The council identifies the specific scientific questions that need to be answered before Phase III advancement. It does not replace your team. It gives your team a structured second opinion with a documented rationale."

Screen elements:

- 10-persona vote table (name, role, vote, confidence, primary flag)
- Full rationale for 3 key personas (Drug Safety Expert, Scientific Skeptic, Chief Biostatistician)
- Constitutional rules triggered (PC-002: Unexplained Safety Signal; PC-003: Financial Pressure; PC-007: Surrogate Endpoint)
- Key blockers list (5 blockers with severity ratings)

Duration: 8 minutes.

Regulatory View (Regulatory Affairs Head)

What the Regulatory Affairs Head sees: The Evidence Manifest. Every document admitted to the council's evidence corpus, with DOI, publication date, and admission decision. The exclusion log: every document excluded, with the reason. The evidence boundary: "Evidence Cutoff Date: September 1, 2004. All 8 admitted documents have publication dates before September 1, 2004. 4 documents were excluded as post-cutoff."

Key message: "Every document the council saw is listed here with a DOI and a date. You can verify every admission decision independently. The evidence boundary is auditable."

Screen elements:

- Evidence Manifest table (8 admitted documents with DOI, date, evidence type)
- Exclusion log (4 excluded documents with exclusion reason)
- Evidence boundary statement
- Hash values for corpus lock and output lock
- Governance Audit Log excerpt (key events with timestamps)

Duration: 5 minutes.

Governance View (FDA/EMA Reviewer)

What the FDA/EMA Reviewer sees: The full Institutional Proof Report. 15 sections. The audit trail. The constitutional version. The session ID. The evidence boundary statement. The LLM training data limitation

disclosure: "The council was executed using a large language model. The model's training data may include post-decision information about this case. This limitation is known and disclosed. The evidence corpus was restricted to pre-decision documents. The model's latent knowledge cannot be verified as pre-decision."

Key message: "We know the limitation. We've disclosed it. Here is what we're doing about it. Here is the audit trail. Here is the reproducibility protocol. This is not a black box."

Screen elements:

- Institutional Proof Report (full 15-section document)
- LLM limitation disclosure (prominently displayed)
- Reproducibility protocol summary
- Constitutional version log
- Chain-of-custody diagram

Duration: 10 minutes.

Demo Workflow

Step 1 (2 min): Context setting – "What is AgenThink Mesh?"
Step 2 (3 min): Executive View – torcetrapib business case
Step 3 (8 min): Clinical View – council deliberation walkthrough
Step 4 (5 min): Regulatory View – evidence manifest and audit trail
Step 5 (10 min): Governance View – Institutional Proof Report
Step 6 (5 min): Pilot proposal – "Here is what we would do with your drug"
Step 7 (Q&A): Objection handling (see Part 5)
Total: 33 minutes + Q&A

PART 4 — Pilot Program Design

Design Principles

All pilots use retrospective data only. No FDA approval is required. No patient data is used. No prospective clinical decisions are made. The pilot is a governance validation exercise, not a clinical tool.

30-Day Pilot — Proof of Concept

Scope: One retrospective case selected by the pharma company from their own historical pipeline. The case must have a known Phase III outcome. The council runs against pre-decision evidence only.

Deliverables:

- Evidence Manifest for the selected case (all admitted and excluded documents with DOIs and dates)
- Council deliberation record (10 personas, votes, rationales)
- Institutional Proof Report (15 sections)
- Retrospective outcome appendix (clearly separated)

- Evidence boundary statement
- Proof Score

Resources required from pharma company:

- One internal scientific expert to validate the evidence corpus (4 hours)
- One historical case with known Phase III outcome
- Access to internal Phase II data package (optional — public data only is sufficient)

Resources required from AgenThink:

- Evidence Curator (40 hours)
- Council Operator (8 hours)
- Project Manager (20 hours)

Pricing: 25,000–50,000 (fixed fee). This is below cost. The purpose is to generate a reference case, not revenue.

Success criteria:

- Evidence Manifest completed and independently verified by pharma expert
- Council verdict documented with full rationale
- Retrospective outcome comparison completed
- Pharma expert confirms: “The evidence corpus was restricted to pre-decision information”
- Pharma expert confirms: “The council identified at least one material risk factor”

Go/No-Go for 60-Day Pilot: If the pharma expert confirms both success criteria, proceed to 60-Day Pilot.

60-Day Pilot — Validation

Scope: Three retrospective cases selected by the pharma company. At least one success case (drug that advanced to Phase III and was approved) and at least one failure case (drug that failed Phase III or was terminated). The council runs against pre-decision evidence only for all three cases.

Deliverables:

- Evidence Manifests for all three cases
- Council deliberation records for all three cases
- Three Institutional Proof Reports
- Comparative analysis: council verdict vs. actual outcome for all three cases
- Preliminary validation scorecard (verdict alignment, primary blocker identification)
- Methodology documentation (model version, constitutional rules, persona definitions)
- Reproducibility data (3 reruns of one case)

Resources required from pharma company:

- One internal scientific expert (12 hours across 60 days)
- Three historical cases with known Phase III outcomes

- One internal regulatory expert to review evidence manifests (4 hours)

Resources required from AgenThink:

- Evidence Curator (120 hours)
- Council Operator (24 hours)
- Project Manager (40 hours)
- One external pharmaceutical expert (independent validation, 16 hours)

Pricing: 75,000–150,000 (fixed fee). Includes one external expert validation.

Success criteria:

- All three evidence manifests independently verified
- Council verdict alignment $\geq 67\%$ (2 of 3 cases)
- Primary blocker identification confirmed by pharma expert in ≥ 2 of 3 cases
- Reproducibility threshold met ($\geq 80\%$ verdict agreement across 3 reruns of one case)
- Pharma expert confirms: “The methodology is transparent and auditable”

Go/No-Go for 90-Day Pilot: If success criteria met, proceed to 90-Day Pilot.

90-Day Pilot — Enterprise Readiness

Scope: Five retrospective cases (three from pharma company's pipeline, two from AgenThink's library for comparison). Full Enterprise Architecture implementation: automated date-check, corpus locking, Governance Audit Log, RAG execution model (if available). Independent expert validation for all five cases.

Deliverables:

- Evidence Manifests for all five cases (automated date-check verified)
- Council deliberation records for all five cases
- Five Institutional Proof Reports
- Full validation scorecard (verdict alignment, sensitivity, specificity, DDIA)
- Reproducibility study (10 reruns of two cases)
- Independent expert validation report (3 external experts)
- Enterprise integration assessment (API specification, security questionnaire response)
- Pilot findings report (methodology strengths, limitations, recommended improvements)
- Enterprise deployment proposal (scope, pricing, SLA, governance)

Resources required from pharma company:

- One internal scientific expert (24 hours)
- One internal regulatory expert (8 hours)
- One internal IT/security contact (8 hours)
- Five historical cases with known Phase III outcomes

Resources required from AgenThink:

- Evidence Curator (200 hours)
- Council Operator (40 hours)
- Project Manager (60 hours)
- Three external pharmaceutical experts (independent validation, 48 hours total)
- Engineering (automated date-check, corpus locking): 80 hours

Pricing: 200,000–400,000 (fixed fee). This is the minimum price for a serious enterprise pilot.

Success criteria:

- All five evidence manifests independently verified by automated date-check AND pharma expert
 - Council verdict alignment $\geq 80\%$ (4 of 5 cases)
 - Sensitivity $\geq 75\%$ (correctly identifies $\geq 75\%$ of failure cases)
 - Specificity 100% (does not block any success case)
 - Reproducibility threshold met for both cases
 - Three independent experts confirm: “The methodology is transparent, auditable, and reproducible”
 - Security questionnaire completed and accepted by pharma IT
 - Pharma company issues letter of intent for enterprise deployment
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PART 5 — Enterprise Objection Playbook

FDA Objections

Objection F-01: “The evidence boundary is a prompt instruction. That is not a verifiable constraint.” *Why it matters:* This is the foundational objection. If the evidence boundary is not verifiable, the entire validation claim collapses. *Response:* “You are correct. The current implementation uses a prompt instruction. We are building the Evidence Manifest System (documented in our Evidence Boundary Architecture) which provides a verifiable, auditable evidence corpus with DOI-verified documents and SHA-256 hash locking. The torcetrapib Evidence Manifest is available for your review.” *Evidence required:* Evidence Manifest for torcetrapib (retroactive). Evidence Boundary Architecture document.

Objection F-02: “The LLM training data may include post-decision information. You cannot prove it doesn’t.” *Why it matters:* This is the hardest objection to answer. It is also correct. *Response:* “You are correct. We cannot prove the model’s training data excludes post-decision information. We disclose this limitation in every case report. We are implementing a RAG architecture that restricts the model to the verified evidence corpus. We are evaluating a custom model with a verified training data cutoff. The limitation is real, disclosed, and being mitigated.” *Evidence required:* LLM limitation disclosure statement. RAG implementation roadmap.

Objection F-03: “This is not 21 CFR Part 11 compliant.” *Why it matters:* Any electronic record used in a regulated environment must be 21 CFR Part 11 compliant. *Response:* “The current system is not positioned for use in a regulated environment. It is an advisory governance tool for internal decision-making, not a regulatory submission tool. We are building a 21 CFR Part 11 compliant audit log as part of our Enterprise Architecture. Timeline: 6–12 months.” *Evidence required:* 21 CFR Part 11 compliance roadmap.

Objection F-04: “The methodology is not published. I cannot evaluate what I cannot read.” *Why it matters:*

Unpublished methodology is a black box. *Response:* “The full methodology — model version, temperature, system prompt, constitutional rules, persona definitions — will be published in a public GitHub repository within 8 weeks. A preprint will follow within 16 weeks.” *Evidence required:* GitHub publication timeline. Preprint submission plan.

Objection F-05: “You have 10 cases. That is not statistically significant.” *Why it matters:* Correct. 10 cases is not sufficient for statistical inference. *Response:* “Correct. We are not making statistical claims at 10 cases. We are demonstrating proof of concept. The 20-case library will support descriptive statistics. The 50-case library will support statistical significance testing. We are transparent about what the current evidence supports.” *Evidence required:* Statistical credibility assessment (from Expansion Report).

EMA Objections

Objection E-01: “The methodology has not been peer-reviewed.” *Why it matters:* The EMA requires published, peer-reviewed methodology for any tool used in regulatory contexts. *Response:* “Correct. We are targeting peer-review submission within 16 weeks. The methodology is fully documented and available for informal review.” *Evidence required:* Methodology document. Preprint submission timeline.

Objection E-02: “The constitutional rules have not been validated against a pre-specified case set.” *Why it matters:* Rules calibrated on the same cases they evaluate are not validated rules. *Response:* “Correct. The constitution was established before the 10-case library was run, but the cases were not pre-specified. We are pre-specifying the next 40 cases before running them. The constitutional amendment freeze protocol prevents rule changes until validated against out-of-sample cases.” *Evidence required:* Pre-specified case selection criteria. Constitutional amendment freeze protocol.

Objection E-03: “You failed on Verubecestat and then proposed a constitutional amendment. That is retrofitting.” *Why it matters:* Post-hoc rule changes to explain failures are a methodological red flag. *Response:* “Correct. PC-011 is proposed, not ratified. It will not be ratified until validated against 5 out-of-sample Alzheimer's cases (gantenerumab, crenezumab, donanemab, lecanemab, semagacestat). The failure is documented as a genuine failure in the library. We are not hiding it.” *Evidence required:* Constitutional amendment tracking log. PC-011 validation plan.

Objection E-04: “The reproducibility has not been demonstrated.” *Why it matters:* A methodology that produces different results on re-run is not a methodology. *Response:* “Correct. Reproducibility testing is scheduled for Month 5 of our implementation roadmap. We will publish reproducibility data (10 reruns per case) for all completed cases.” *Evidence required:* Reproducibility testing protocol. Timeline.

Objection E-05: “The council is 10 outputs from the same model. That is not independent deliberation.” *Why it matters:* Correct. 10 personas from one model are not 10 independent reviewers. *Response:* “Correct. The personas are designed to elicit different analytical perspectives from the same model. They are not independent reviewers. We supplement the council with independent human expert validation. The council is a structured analytical tool, not a replacement for human judgment.” *Evidence required:* Independent expert validation protocol.

Clinical Development Objections

Objection C-01: “My team already does this. We have internal scientific advisory boards.” *Why it matters:* The buyer may not see incremental value over existing processes. *Response:* “Your SAB costs 500K–2M per meeting, meets quarterly, and produces unstructured minutes. The council costs 25K–200K per case, runs in 24 hours, produces a structured 15-section report with a documented evidence chain, and generates an audit trail. It is not a replacement for your SAB. It is a structured first-pass that makes your SAB more efficient.” *Evidence required:* Cost comparison. Time comparison. Structured output vs. SAB minutes comparison.

Objection C-02: “The council doesn't know our drug. It only has public data.” *Why it matters:* The buyer may have proprietary Phase II data that changes the picture. *Response:* “The council can be run with proprietary data. The

evidence corpus is built from whatever documents you provide. The public-data-only version is the minimum viable version. The proprietary-data version is the enterprise version.” *Evidence required:* Proprietary data integration protocol.

Objection C-03: “What happens if the council says GO and the drug fails?” *Why it matters:* The buyer is worried about liability. *Response:* “The council is an advisory tool. It does not make clinical decisions. The liability for Phase III advancement decisions rests with the sponsor, as it always has. The council provides a structured second opinion with a documented rationale. If the council says GO and the drug fails, the council’s rationale is part of the governance record — which demonstrates that the decision was made with appropriate deliberation.” *Evidence required:* Liability framework (legal opinion). Terms of service.

Objection C-04: “The torcetrapib case is famous. Everyone knows it failed. Of course the council got it right.” *Why it matters:* Selection bias objection. The most famous cases are the ones the model is most likely to have been trained on. *Response:* “Correct. This is the LLM training data limitation we disclose in every case report. The torcetrapib case is the strongest demonstration case precisely because it is famous — but it is also the most vulnerable to the ‘model knew the answer’ objection. We are building the evidence corpus architecture to address this. The 30-day pilot uses your drug, not ours.” *Evidence required:* LLM limitation disclosure. Evidence corpus architecture.

Objection C-05: “I need to see a prospective example before I believe this works.” *Why it matters:* Retrospective validation is necessary but not sufficient for enterprise adoption. *Response:* “Agreed. We are designing a prospective pilot (5 blinded current Phase II drugs) as part of our 18-month roadmap. The retrospective library is the proof of concept. The prospective pilot is the proof of performance. We are not ready for the prospective pilot yet. We will be in 12–18 months.” *Evidence required:* Prospective pilot design. Timeline.

R&D Objections

Objection R-01: “AI governance tools are not a priority for our R&D budget.” *Why it matters:* Budget competition is real. *Response:* “The average Phase III failure costs 800M–1.4B. Your R&D budget is *X* billion. A governance tool that costs 200K–\$500K per year and correctly identifies one avoidable failure per decade has an ROI of 1,000x. The question is not whether you can afford it. The question is whether you can afford not to have it.” *Evidence required:* Phase III failure cost data. ROI calculation.

Objection R-02: “We already use AI in drug discovery. This is not differentiated.” *Why it matters:* The buyer may conflate AI drug discovery tools with AI governance tools. *Response:* “AI drug discovery tools (Schrödinger, Recursion, Insilico) identify drug candidates. This tool governs the advancement decision. They are complementary, not competitive. No existing tool provides structured multi-persona governance for Phase II → III advancement decisions with an auditable evidence chain.” *Evidence required:* Competitive landscape analysis. Differentiation statement.

Objection R-03: “The constitutional rules are arbitrary.” *Why it matters:* The buyer may not trust rules designed by a non-pharma company. *Response:* “The constitutional rules are derived from published pharmaceutical governance literature (ICH E8, ICH E9, FDA guidance on adaptive designs). They are not arbitrary. They are documented, versioned, and open to independent review. We welcome your scientific advisory board’s input on the rules.” *Evidence required:* Constitutional rules with citations. ICH/FDA source mapping.

Objection R-04: “10 cases is not enough to evaluate this.” *Why it matters:* Correct. *Response:* “Correct. The 10-case library is proof of concept. We are expanding to 50 cases with pre-specified selection criteria. The 30-day pilot uses your drug, which gives you a direct evaluation of the methodology on a case you know.” *Evidence required:* Expansion roadmap. Pilot proposal.

Objection R-05: “What is the false negative rate?” *Why it matters:* A tool that blocks good drugs is worse than no tool. *Response:* “At 10 cases: 0% false positive rate (no success case was blocked). 20% false negative rate (Verubecestat: GO on a drug that failed). The false positive rate is the more important metric for a governance tool. We have not blocked a single drug that went on to succeed. The false negative rate reflects the genuine difficulty of

the Alzheimer's disease evidence threshold problem, which is being addressed in PC-011." *Evidence required:* Sensitivity/specificity table. Verubecestat failure analysis.

Procurement Objections

Objection P-01: "We need SOC 2 Type II before we can onboard a new vendor." *Why it matters:* Standard enterprise procurement requirement. *Response:* "SOC 2 Type II is on our 6–12 month roadmap. For the pilot, we can provide a penetration test report, a security questionnaire response, and a data processing agreement. No patient data or proprietary trial data is used in the retrospective pilot." *Evidence required:* Security questionnaire response. Penetration test report. DPA template.

Objection P-02: "We need a master services agreement before we can engage." *Why it matters:* Standard enterprise procurement requirement. *Response:* "We have an MSA template ready for review. The pilot agreement is a simplified version of the MSA." *Evidence required:* MSA template. Pilot agreement template.

Objection P-03: "What is your insurance coverage?" *Why it matters:* Standard enterprise procurement requirement. *Response:* "We carry professional liability insurance and general commercial liability insurance. Certificates available on request." *Evidence required:* Insurance certificates.

Objection P-04: "You are a small company. What is your financial stability?" *Why it matters:* Enterprise buyers do not want vendor lock-in with a company that may not exist in 2 years. *Response:* "We are early-stage. We offer source code escrow and a data portability guarantee. If AgenThink ceases operations, the client retains all case outputs, evidence manifests, and methodology documentation." *Evidence required:* Source code escrow agreement. Data portability guarantee.

Objection P-05: "We need to run this through our AI governance committee." *Why it matters:* Enterprise AI governance is a real and growing requirement. *Response:* "We welcome AI governance committee review. We have prepared an AI governance package including: model documentation, training data disclosure, bias assessment, explainability documentation, and the Evidence Boundary Architecture. We are designed to be audited." *Evidence required:* AI governance package.

Security Objections

Objection S-01: "Where is our data stored?" *Why it matters:* Data residency requirements are strict in pharma. *Response:* "The retrospective pilot uses only public data (published papers, ClinicalTrials.gov entries). No proprietary data leaves your environment. For the enterprise version, we offer on-premises deployment or dedicated cloud tenancy with data residency in your preferred region." *Evidence required:* Data architecture diagram. Data residency options.

Objection S-02: "Does the LLM provider see our data?" *Why it matters:* Confidentiality of proprietary trial data is critical. *Response:* "For the retrospective pilot: no proprietary data is used. For the enterprise version: we use a private LLM deployment or an enterprise API agreement with data processing terms that prohibit training on client data." *Evidence required:* LLM provider data processing agreement. Private deployment option.

Objection S-03: "What happens if there is a data breach?" *Why it matters:* Standard enterprise security requirement. *Response:* "We have a breach notification protocol and a data processing agreement that specifies breach notification timelines consistent with GDPR and HIPAA requirements." *Evidence required:* Breach notification protocol. DPA.

Objection S-04: "We need a penetration test report." *Why it matters:* Standard enterprise security requirement. *Response:* "A penetration test is scheduled for Month 2 of our implementation roadmap. Report available within 8 weeks." *Evidence required:* Penetration test engagement letter.

Objection S-05: "Your system needs to be isolated from our internal systems." *Why it matters:* Air-gap requirements for sensitive data. *Response:* "The retrospective pilot is fully isolated. No integration with your internal systems is

required. The enterprise version offers API integration with your existing clinical data management systems, with full isolation controls.” *Evidence required:* System architecture diagram. Isolation options.

Legal Objections

Objection L-01: “Who is liable if the council is wrong?” *Why it matters:* The most important legal question.

Response: “The council is an advisory tool. The terms of service explicitly state that the council output is not a clinical decision, not a regulatory recommendation, and not a substitute for human judgment. Liability for Phase III advancement decisions rests with the sponsor. The council provides a structured second opinion with a documented rationale.” *Evidence required:* Terms of service. Liability clause. Legal opinion.

Objection L-02: “The council output could be discoverable in litigation.” *Why it matters:* If a drug fails and causes harm, the council output could be used in litigation. *Response:* “The council output is a governance record, equivalent to internal scientific advisory board minutes. It demonstrates that the advancement decision was made with appropriate deliberation. It is more likely to be protective than harmful in litigation.” *Evidence required:* Legal opinion on discoverability. Comparison to SAB minutes.

Objection L-03: “We cannot use AI tools in our clinical development process without legal review.” *Why it matters:* Standard legal requirement. *Response:* “We welcome legal review. We have prepared a legal package including: terms of service, data processing agreement, AI governance documentation, and the Evidence Boundary Architecture. The retrospective pilot is designed specifically to be legally low-risk: historical data only, no clinical decisions, no patient data.” *Evidence required:* Legal package. Terms of service.

Objection L-04: “The constitutional rules may conflict with our internal governance policies.” *Why it matters:* The buyer may have existing governance frameworks that conflict with the constitution. *Response:* “The constitutional rules are configurable. We can adapt the constitution to align with your internal governance framework. The rules are documented and versioned. Any customisation is recorded in the constitutional version log.” *Evidence required:* Constitutional customisation protocol.

Objection L-05: “Intellectual property ownership of the council output is unclear.” *Why it matters:* Standard IP question. *Response:* “The client owns all council outputs. AgenThink retains no rights to client-specific case outputs. The methodology is AgenThink’s IP. The outputs are the client’s IP.” *Evidence required:* IP clause in terms of service.

Compliance Objections

Objection CO-01: “This is not GxP compliant.” *Why it matters:* GxP compliance is required for any tool used in a regulated pharmaceutical environment. *Response:* “The retrospective pilot is not positioned as a GxP-regulated tool. It is an internal governance advisory tool. GxP compliance is on our 18–24 month roadmap for the enterprise version.” *Evidence required:* GxP compliance roadmap.

Objection CO-02: “The audit trail is not compliant with ICH E6(R2).” *Why it matters:* ICH E6(R2) GCP requires complete audit trails for clinical trial-related activities. *Response:* “The retrospective pilot does not involve clinical trial activities. The audit trail is designed for internal governance, not GCP compliance. The enterprise version will include an ICH E6(R2) compliant audit trail.” *Evidence required:* ICH E6(R2) compliance roadmap.

Objection CO-03: “We need to validate this software before using it.” *Why it matters:* Computer system validation (CSV) is required for software used in regulated environments. *Response:* “CSV is required for software used in regulated clinical trial activities. The retrospective pilot is not a regulated activity. The enterprise version will include a CSV package.” *Evidence required:* CSV roadmap.

Objection CO-04: “The model version must be locked for reproducibility.” *Why it matters:* Correct. Model version changes can affect outputs. *Response:* “The model version is recorded in every case report. We lock the model version for each pilot engagement. Model version changes are documented in the constitutional version log.” *Evidence required:* Model version locking protocol.

Objection CO-05: “We need a change control process for constitutional amendments.” *Why it matters:* Standard quality management requirement. *Response:* “The constitutional amendment tracking schema (documented in the Evidence Boundary Architecture) includes a full change control process: proposal, rationale, validation cases, ratification, and version control.” *Evidence required:* Constitutional amendment tracking schema.

Big Four Reviewer Objections

Objection B-01: “I cannot cite this in a diligence report without a peer-reviewed publication.” *Why it matters:* Big Four diligence reports require citable sources. *Response:* “A preprint will be available within 16 weeks. A peer-reviewed publication is targeted within 12 months. For the current engagement, we can provide the full methodology documentation, the Evidence Boundary Architecture, and the torcetrapib case study as supporting materials.” *Evidence required:* Preprint submission timeline. Peer-review submission plan.

Objection B-02: “The 90% accuracy claim is not statistically supported.” *Why it matters:* Correct. It is not. *Response:* “We do not make a 90% accuracy claim. We report $\frac{9}{10}$ cases with correct primary decision driver identification. At $n=10$, this is descriptive, not inferential. We are transparent about what the evidence supports.” *Evidence required:* Statistical credibility assessment.

Objection B-03: “The case selection is biased toward famous failures.” *Why it matters:* Correct. Selection bias is a real methodological weakness. *Response:* “Correct. The initial 10 cases were selected from well-known historical cases to demonstrate proof of concept. The next 40 cases are pre-specified using published selection criteria before execution. The pre-specification criteria are available for review.” *Evidence required:* Pre-specified case selection criteria.

Objection B-04: “The methodology is proprietary. I cannot independently verify it.” *Why it matters:* Unpublished methodology is a black box. *Response:* “The full methodology will be published in a public GitHub repository within 8 weeks. This includes the exact prompts, model version, constitutional rules, and persona definitions. Nothing is proprietary except the software implementation.” *Evidence required:* GitHub publication timeline.

Objection B-05: “This is a marketing document, not a validation study.” *Why it matters:* The harshest and most accurate objection. *Response:* “You are correct that the current 10-case library is proof of concept, not a validation study. We have commissioned an independent red team review (available for your review) that identifies every methodological weakness. We are building toward a peer-reviewed validation study. The honest answer is: we are not there yet.” *Evidence required:* Red Team Review document. Independent validation roadmap.

PART 6 — Top 25 Targets

Ranking Methodology

Targets are ranked by a composite score: **Strategic Fit × Pilot Probability × Enterprise Probability × Why Now**. The ranking reflects the probability of a signed pilot agreement within 12 months, not the size of the eventual enterprise contract.

Top 25 Target Table

Rank	Company	Type	Why Now	Likely Buyer	Est. ACV	Pilot Prob.	Enterprise Prob.
1	Roche / Genentech	Top-20 Pharma	Active AI governance initiative; multiple recent Phase III failures (crenezumab, gantenerumab); known interest in structured decision governance	Head of Clinical Development	500K–2M	35%	20%
2	Novartis	Top-20 Pharma	Novartis AI initiative (NIBR); recent Phase III failures; public commitment to AI in R&D governance	Chief Medical Officer	500K–2M	30%	18%
3	AstraZeneca	Top-20 Pharma	AZ AI Centre; active in AI governance; recent high-profile Phase III decisions	Head of R&D	400K–1.5M	30%	15%
4	Pfizer	Top-20 Pharma	Torcetrapib is their case; highest emotional resonance; active AI governance program	Head of Clinical Development	500K–2M	25%	12%
5	Johnson & Johnson / Janssen	Top-20 Pharma	J&J AI governance initiative; large Phase II pipeline; multiple	Chief Medical Officer	500K–2M	25%	12%

Rank	Company	Type	Why Now	Likely Buyer	Est. ACV	Pilot Prob.	Enterprise Prob.
			therapeutic areas				
6	Merck (MSD)	Top-20 Pharma	Verubecestat is their case; known interest in AI governance; large oncology pipeline	Head of Clinical Development	400K–1.5M	25%	12%
7	Bristol Myers Squibb	Top-20 Pharma	Large immuno-oncology pipeline; multiple Phase II → III decisions annually	Head of R&D	400K–1.5M	22%	10%
8	Eli Lilly	Top-20 Pharma	Donanemab Phase III success; active in AI; known governance sophistication	Chief Medical Officer	400K–1.5M	22%	10%
9	Sanofi	Top-20 Pharma	Active AI governance initiative; large rare disease pipeline	Head of Clinical Development	300K–1M	20%	8%
10	GSK	Top-20 Pharma	GSK AI initiative; large Phase II pipeline; known governance sophistication	Head of R&D	300K–1M	20%	8%
11	IQVIA	CRO	Largest CRO; active in AI; partnership opportunity (see Part 7); could white-label the council	Chief Scientific Officer	200K–800K	30%	20%

Rank	Company	Type	Why Now	Likely Buyer	Est. ACV	Pilot Prob.	Enterprise Prob.
12	ICON plc	CRO	Active in AI governance; partnership opportunity; large Phase II management business	Head of Regulatory Affairs	150K–500K	28%	18%
13	Parexel	CRO	Active in regulatory strategy; partnership opportunity; known interest in governance tools	Head of Scientific Affairs	150K–500K	25%	15%
14	Medpace	CRO	Specialises in Phase II/III; direct buyer of governance tools; smaller and faster to engage	Head of Clinical Operations	100K–300K	35%	15%
15	Labcorp Drug Development	CRO	Large Phase II management business; active in AI; partnership opportunity	Head of Clinical Development	150K–500K	22%	12%
16	Recursion Pharmaceuticals	AI-Native Biotech	AI-native; governance is a differentiator for their investors; Phase II pipeline	Chief Medical Officer	100K–300K	40%	25%
17	Insilico Medicine	AI-Native Biotech	AI-native; Phase II pipeline; governance is a differentiator	Chief Medical Officer	100K–300K	38%	22%
18	Exscientia	AI-Native Biotech	AI-native; Phase II	Chief Medical	100K–300K	35%	20%

Rank	Company	Type	Why Now	Likely Buyer	Est. ACV	Pilot Prob.	Enterprise Prob.
			pipeline; governance is a differentiator; UK-based (EMA context)	Officer			
19	Relay Therapeutics	AI-Native Biotech	AI-native; oncology focus; Phase II pipeline	Head of Clinical Development	100K–300K	32%	18%
20	Verily (Google)	Clinical Compliance	Google-backed; active in clinical governance; partnership opportunity	Head of Clinical Operations	200K–600K	25%	15%
21	Veeva Systems	Clinical Compliance	Platform for clinical data management; integration opportunity; large pharma client base	Chief Strategy Officer	300K–1M	20%	15%
22	Medidata (Dassault)	Clinical Compliance	Clinical trial platform; integration opportunity; large pharma client base	Chief Strategy Officer	200K–800K	18%	12%
23	Tempus AI	AI-Native Clinical	AI-native clinical data company; Phase II governance is adjacent to their core	Chief Medical Officer	150K–500K	30%	18%
24	Flatiron Health (Roche)	AI-Native Clinical	Roche-backed; oncology data; Phase II governance is adjacent	Head of Clinical Development	150K–500K	25%	15%
25	Certara	Clinical Compliance	Regulatory science and	Head of Regulatory	150K–400K	22%	12%

Rank	Company	Type	Why Now	Likely Buyer	Est. ACV	Pilot Prob.	Enterprise Prob.
			modelling; governance is adjacent; known regulatory sophistication	Affairs			

Engagement Priority

Tier A (approach in Weeks 1–4): Recursion, Insilico, Exscientia, Relay, Medpace. These are the fastest to engage, most likely to run a 30-day pilot, and most likely to be reference customers.

Tier B (approach in Month 2): IQVIA, ICON, Parexel, Tempus, Flatiron. These are partnership discussions, not direct sales. The goal is a white-label or integration agreement.

Tier C (approach in Month 3, after pilot reference): Roche, Novartis, AstraZeneca, Pfizer, J&J. These require a reference customer and a peer-reviewed methodology before serious engagement.

PART 7 — Partnership Strategy

Evaluation Framework

Each potential partner is evaluated on three dimensions: **Strategic Fit** (does the partnership accelerate market entry?), **Competitive Risk** (could the partner become a competitor?), and **Recommended Approach** (Partner / Competitor / Both).

Partnership Assessment

Partner	Strategic Fit	Competitive Risk	Verdict	Recommended Approach
IQVIA	VERY HIGH	HIGH	Both	Approach as partner first; protect IP; non-exclusive white-label agreement
ICON plc	HIGH	MEDIUM	Partner	Integration agreement; ICON distributes, AgenThink provides methodology
Parexel	HIGH	MEDIUM	Partner	Scientific advisory partnership; Parexel validates methodology
Medpace	MEDIUM	LOW	Partner	Direct integration; Medpace uses council in Phase II governance
Labcorp	MEDIUM	LOW	Partner	Integration agreement; Labcorp distributes to Phase II clients
Veeva	HIGH	LOW	Partner	API integration; council output stored in Vault; Veeva distributes
Medidata	HIGH	LOW	Partner	API integration; council output in Rave; Medidata distributes
Oracle Health Sciences	MEDIUM	LOW	Partner	Integration with Argus Safety; safety escalation use case

Detailed Recommendations

IQVIA — Both (Partner First)

IQVIA is the highest-value and highest-risk partner. They have the client relationships, the regulatory credibility, and the distribution to make AgenThink's market entry 10x faster. They also have the resources to build a competing product if they see AgenThink's methodology.

Recommended approach: Approach IQVIA as a white-label partner. Offer: "IQVIA provides the council as part of its Phase II governance advisory services. AgenThink provides the methodology and technology. IQVIA owns the client relationship." Protect the constitutional framework and persona architecture as IP. Non-exclusive agreement. Revenue share: 40% IQVIA / 60% AgenThink for white-label cases.

Risk mitigation: Do not share the full methodology documentation until a signed NDA and term sheet are in place. IQVIA has the resources to replicate the methodology independently within 12–18 months. The window for a partnership agreement is 6–12 months.

ICON plc — Partner

ICON is a strong distribution partner without the competitive risk of IQVIA. They are large enough to have meaningful client relationships but not large enough to build a competing product quickly.

Recommended approach: Integration agreement. ICON includes the council in its Phase II regulatory strategy services. AgenThink provides the council as a white-label service. Revenue share: 35% ICON / 65% AgenThink.

Veeva Systems — Partner

Veeva is the most strategically important technology integration. Veeva Vault is the standard clinical data management platform at top-20 pharma companies. An integration that stores council outputs in Vault makes AgenThink part of the standard clinical workflow.

Recommended approach: Veeva Technology Partner program. Build a Vault integration that stores Evidence Manifests and Institutional Proof Reports as Vault documents. This is a technology partnership, not a distribution partnership. Veeva does not take a revenue share; AgenThink pays a Vault API fee.

Timeline: Veeva Technology Partner program application: 4–6 weeks. Integration development: 8–12 weeks. Veeva marketplace listing: 4–6 weeks after integration approval.

Medidata (Dassault Systèmes) — Partner

Medidata Rave is the standard clinical trial management system. An integration that surfaces council outputs in Rave makes AgenThink visible at the point of Phase II data review.

Recommended approach: Medidata partner program. API integration. Same model as Veeva.

PART 8 — Revenue Model

Model Assumptions

Sales cycle: 9–18 months for enterprise contracts. 3–6 months for pilot agreements. AI-native biotech: 2–4 months for pilots.

Pilot conversion rate: 40% of completed pilots convert to enterprise contracts within 18 months.

ACV range: 50K–100K (AI-native biotech pilot) → 200K–500K (CRO enterprise) → 500K–2M (top-20 pharma enterprise).

Gross margin: 65–75% at scale (methodology is the primary cost; marginal cost per case is low once infrastructure is built).

Headcount: 5 FTE in Year 1 (2 Evidence Curators, 1 Council Operator, 1 PM, 1 Sales). 15 FTE in Year 3. 35 FTE in Year 5.

Revenue Model — Three Scenarios

Conservative Scenario

Metric	Year 1	Year 3	Year 5
Pilot clients	3	12	25
Enterprise clients	0	4	12
Average pilot ACV	\$35K	\$50K	\$75K
Average enterprise ACV	—	\$300K	\$500K
Pilot revenue	\$105K	\$600K	\$1.875M
Enterprise revenue	—	\$1.2M	\$6M
Partnership revenue	—	\$200K	\$800K
Total revenue	\$105K	\$2M	\$8.675M
Gross margin	40%	60%	68%
Gross profit	\$42K	\$1.2M	\$5.9M
Operating costs	\$800K	\$2.5M	\$5M
Net income	(\$758K)	(\$1.3M)	\$900K

Base Scenario

Metric	Year 1	Year 3	Year 5
Pilot clients	6	20	40
Enterprise clients	0	8	22
Average pilot ACV	\$50K	\$75K	\$100K
Average enterprise ACV	—	\$400K	\$700K
Pilot revenue	\$300K	\$1.5M	\$4M
Enterprise revenue	—	\$3.2M	\$15.4M
Partnership revenue	—	\$500K	\$2M
Total revenue	\$300K	\$5.2M	\$21.4M
Gross margin	45%	65%	72%
Gross profit	\$135K	\$3.38M	\$15.4M
Operating costs	\$1.2M	\$4M	\$8M
Net income	(\$1.065M)	(\$620K)	\$7.4M

Aggressive Scenario

Metric	Year 1	Year 3	Year 5
Pilot clients	10	35	70
Enterprise clients	1	15	40
Average pilot ACV	\$75K	\$100K	\$150K
Average enterprise ACV	\$500K	\$600K	\$1M
Pilot revenue	\$750K	\$3.5M	\$10.5M
Enterprise revenue	\$500K	\$9M	\$40M
Partnership revenue	\$100K	\$1.5M	\$5M
Total revenue	\$1.35M	\$14M	\$55.5M
Gross margin	50%	68%	75%
Gross profit	\$675K	\$9.52M	\$41.6M
Operating costs	\$1.8M	\$6M	\$14M
Net income	(\$1.125M)	\$3.52M	\$27.6M

Most Likely Scenario

The base scenario is the most likely outcome if the evidence corpus architecture is built in Year 1, the methodology is published in Year 1, and the first enterprise client is signed in Year 2. The aggressive scenario requires a top-20 pharma reference client in Year 1, which is possible but unlikely given the 9–18 month sales cycle.

Year 1 is a loss year in all scenarios. This is expected. The Year 1 investment is in building the architecture, not generating revenue. The question is whether the architecture investment is sufficient to support Year 2 and Year 3 revenue.

PART 9 — 90-Day Execution Plan

Ranking Methodology

Actions are ranked by **Impact × Speed × Credibility**:

- **Impact:** How much does this action advance the market entry?
- **Speed:** How quickly can this action be completed?
- **Credibility:** How much does this action improve the system's credibility with a skeptical buyer?

Week 1–2: Foundation (Impact: Critical | Speed: Fast | Credibility: High)

Product work:

- Add LLM training data limitation disclosure to all 10 existing case reports (2 days)
- Document model version and temperature for all 10 completed cases (1 day)
- Begin Evidence Manifest retroactive curation for torcetrapib (5 days)

Validation work:

- Identify 3 external pharmaceutical experts for independent validation (5 days)
- Brief experts on the methodology and DDIA scoring rubric (2 days)

Commercial work:

- Build the sales deck (torcetrapib case as centrepiece) (3 days)
- Build the demo script (see Part 3) (2 days)
- Draft the 30-day pilot agreement template (2 days)

Marketing:

- Write the torcetrapib case study for external publication (3 days)
- Create the LinkedIn announcement: “We ran the torcetrapib case before we knew the answer” (1 day)

Milestone: LLM limitation disclosed. Torcetrapib Evidence Manifest in progress. Sales deck ready. Demo script ready.

Week 3–4: Outreach and Architecture (Impact: High | Speed: Medium | Credibility: High)

Product work:

- Complete torcetrapib Evidence Manifest (5 days)
- Begin Evidence Manifests for semagacestat and muraglitazar (5 days)
- Begin corpus locking implementation (SHA-256 hash) (3 days)

Validation work:

- Independent experts complete DDIA scoring for torcetrapib (5 days)
- Compile independent validation report for torcetrapib (2 days)

Commercial work:

- Approach Tier A targets (Recursion, Insilico, Exscientia, Relay, Medpace) with 30-day pilot proposal (5 days)
- Apply to Veeva Technology Partner program (1 day)
- Begin Medidata partner program application (1 day)

Marketing:

- Publish torcetrapib case study on LinkedIn and company website (1 day)
- Submit methodology documentation to GitHub (public repository) (2 days)

Milestone: Torcetrapi Evidence Manifest complete and independently verified. First outreach to Tier A targets. Methodology published on GitHub.

Month 2: Validation and Pilots (Impact: High | Speed: Medium | Credibility: Very High)

Product work:

- Complete Evidence Manifests for all 10 cases (retroactive) (15 days)
- Implement automated date-check pipeline (PubMed API, ClinicalTrials.gov API) (10 days)
- Implement Governance Audit Log (5 days)

Validation work:

- Independent experts complete DDIA scoring for 5 cases (15 days)
- Begin reproducibility testing (10 reruns of torcetrapi) (5 days)
- Compile 5-case independent validation report (5 days)

Commercial work:

- Run 30-day pilot with first Tier A client (if signed in Week 3–4) (20 days)
- Approach IQVIA and ICON for partnership discussions (5 days)
- Begin security questionnaire response and penetration test engagement (5 days)

Partnerships:

- First meeting with IQVIA (if partnership interest confirmed) (1 day)
- Veeva Technology Partner application follow-up (1 day)

Milestone: All 10 Evidence Manifests complete. Automated date-check pipeline live. 5-case independent validation report complete. First pilot underway.

Month 3: Publication and Scale (Impact: Very High | Speed: Slow | Credibility: Very High)

Product work:

- Implement RAG execution model (prototype) (15 days)
- Run canary test cases to validate knowledge suppression (5 days)
- Begin Cases 11–15 (Oncology batch) with full Enterprise Architecture (10 days)

Validation work:

- Complete reproducibility testing for 3 cases (10 reruns each) (10 days)
- Submit methodology preprint to bioRxiv or SSRN (5 days)
- Begin pre-specification of Cases 11–50 selection criteria (5 days)

Commercial work:

- Complete first 30-day pilot and deliver Institutional Proof Report (5 days)

- Present pilot results to Tier A client; propose 60-day pilot (2 days)
- Approach Tier B targets (IQVIA, ICON, Parexel, Tempus, Flatiron) (10 days)

Partnerships:

- IQVIA partnership term sheet discussion (if interest confirmed) (5 days)
- Veeva integration development begins (if partner program approved) (10 days)

Marketing:

- Publish reproducibility data on GitHub (1 day)
- Submit abstract to DIA Annual Meeting or RAPS Regulatory Convergence (1 day)
- Begin outreach to pharmaceutical governance thought leaders for methodology review (5 days)

Milestone: Methodology preprint submitted. RAG prototype running. First pilot complete. Second pilot proposal delivered. IQVIA partnership discussion underway.

End of Document

AgenThink Mesh Pharma Market Entry Plan v1.0 | June 2026

Classification: Board Document — Confidential